

BENCHMARK™ INTERNATIONAL

PROVIDER SERVICES | LIFE SCIENCES & DIAGNOSTICS | MEDICAL PRODUCTS & DISTRIBUTION | PHARMACY & HEALTHCARE SERVICES

HEALTHCARE

Q4 2026 M&A Sector Report

OCTOBER 2026 - JARED HARDIN | MANAGING DIRECTOR | BENCHMARK INTERNATIONAL

The quarter in brief

Healthcare enters the fourth quarter of 2026 with health systems, distributors and public strategic buyers setting the pace, and with private equity completing fewer and more carefully underwritten transactions. Private equity closed roughly 337 healthcare services deals in the first half, a pace that would make 2026 the slowest year since 2017, while medtech deal value reached $36.5 billion over the same six months and pharma and life sciences recorded its strongest quarter since 2020. Demand underneath the deal market is steady, with national health spending running 6.8% above last year in July and projected to reach nearly $9.0 trillion by 2034.

6.8%
Growth in national health spending in the year to July 2026

337
Private equity healthcare services deals in the first half, the slowest pace since 2017

$36.5B
Medtech deal value in the first half of 2026

13.6x
Median EBITDA multiple private equity paid for healthcare services companies in 2025, weighted toward larger platforms

Information as of October 5, 2026. Public-company and larger-platform multiples are reference points, and a private company's price depends on the business itself.

THE FOUR FOCUS AREAS AT THE START OF THE QUARTER

FOCUS AREA	WHERE THE MARKET STANDS	WHO IS BUYING	BENCHMARK INTERNATIONAL: CLOSED IN THE TRAILING 24 MONTHS, AND LIVE
Provider Services	The United States has 14,655 urgent care centers and the 100 largest operators hold 41% of them; Medicare has proposed lowering the physician conversion factor by 1.2% to 1.7% for 2027	HCA Healthcare with 70 urgent care centers acquired in 2026, MyEyeDr., U.S. Physical Therapy, Addus HomeCare and Concentra	2 closed, 19 live
Life Sciences & Diagnostics	Research outsourcing demand is recovering, with IQVIA booking $1.22 of new work for each dollar of revenue; Medicare laboratory fee cuts of up to 15% a year are scheduled to resume in 2027	Labcorp and Quest for laboratories, Thermo Fisher, Danaher and Merck KGaA for tools and reagents, sponsor-backed clinical research site networks	4 closed, 4 live
Medical Products & Distribution	Stryker grew 9.0% organically and Steris grew healthcare consumables 9%; tariff costs and refunds are moving earnings in both directions, and 41% of health systems plan to cut capital spending	KKR, Danaher, Mitsui Chemicals, Cardinal Health, Aspen Surgical and Platinum Equity	1 closed, 8 live
Pharmacy & Healthcare Services	Net medicine spending reached $606 billion in 2025; negotiated Medicare prices are compressing long-term care pharmacy revenue; healthcare staffing is forecast to grow 1% to $39.0 billion	Guardian Pharmacy, Cencora, Cardinal Health and McKesson, Knox Lane in staffing, Strata Critical Medical in perfusion	5 closed, 8 live

Deal market: strategic buyers are active while private equity completes fewer deals

Private equity healthcare services deal value was $17.8 billion in the first half of 2026, 7.3% below the prior year, and the roughly 337 transactions in the period annualize to 674 against an average of 903 a year between 2018 and 2024 (PitchBook, Q2 2026 Healthcare Services Report; Fierce Healthcare, August 2026). Physician practice management accounts for most of the decline, with 71 deals in the second quarter against 111 a year earlier, while ancillary and outsourced services such as surgery centers, staffing and laboratories are tracking only 4.9% below 2025 (Becker's ASC Review). PwC counted 300 health services transactions in the first quarter and about $28 billion of value through May 31 (PwC via Fierce Healthcare, June 17, 2026).

Deal activity in the rest of healthcare has been stronger than in provider services, and pharma and life sciences deal value exceeded $65 billion in the first quarter, with 16 transactions of $1 billion or more, the strongest quarter since 2020 (PwC via Fierce Biotech). Medtech followed a decade-high 2025 with $36.5 billion of deal value in the first half (PwC via MedTech Dive), and hospitals and health systems announced 18 transactions in the second quarter, including three mega-mergers (Kaufman Hall, July 13, 2026).

HEALTHCARE DEAL INDICATOR	LATEST READING	PERIOD	SOURCE
All healthcare transactions	549 deals, level with both the prior quarter and the prior year; private equity was the buyer in 181	Q1 2026	LevinPro data via Becker's ASC Review
Physician group share of health services deals	46%, up from 37% a year earlier	Q1 2026	PwC via Fierce Healthcare
Median private equity entry multiple, healthcare services	13.6x EBITDA, with median net debt of 5.2x EBITDA	2025	PitchBook via Becker's ASC Review
Projected private equity healthcare services exits	Down 26.5% by count and 30.9% by value	2026	PitchBook
Average purchase price, completed middle-market transactions in all industries	7.0x trailing adjusted EBITDA, with senior debt priced at 7.8%	Q2 2026	GF Data via ACG
Median multiple, transactions of $5 million to $50 million in all industries	5.8x, the highest since early 2022	Q2 2026	IBBA and M&A Source Market Pulse

Financing became more expensive during the third quarter, when the Federal Reserve raised its target range by a quarter point to 3.75% to 4.00% on September 16 (Federal Reserve), the 10-year Treasury yielded 5.24% on October 1 (US Treasury), and direct lenders widened buyout loan spreads to an average of 509 basis points over the benchmark rate in the second quarter from 474 in the first (PitchBook, Q2 2026 US PE Breakdown).

"Financial sponsors still have near-record levels of 'dry powder,' but they are becoming increasingly more selective with the assets they choose to invest in."

Houlihan Lokey, Summer 2026 Payor Technology Deep Dive

Demand: spending and employment continue to grow

National health spending grew 6.8% in the year to July 2026, after a revised 7.1% in June, and represented 18.4% of GDP (Altarum Health Sector Economic Indicators, September 28, 2026). CMS projects nearly $9.0 trillion of spending in 2034, or 20.6% of GDP, on average annual growth of 5.4% (CMS National Health Expenditure fact sheet). Healthcare employers added 17,000 jobs in September against a twelve-month average of 33,000 (Bureau of Labor Statistics, October 2, 2026). Spending growth supports demand across the sector, and a particular business captures it through its payer rates, labor costs and collections.

REIMBURSEMENT, POLICY AND DEAL REGULATION

Medicare rates for 2027 are proposed and the Medicaid changes begin in January

ITEM	STATUS IN OCTOBER 2026	SOURCE
Medicare Physician Fee Schedule, 2027	Proposed conversion factors of $33.17 for qualifying participants in advanced alternative payment models and $32.84 for other clinicians, reductions of 1.19% and 1.68%, as the one-year 2.5% increase for 2026 expires; final rule pending	CMS proposed rule
Hospital outpatient and surgery center rates, 2027	Proposed increase of 2.4% for both; 637 procedures would leave the inpatient-only list and about 618 would be added to the surgery center list	CMS rule register
Home health, 2027	Proposed aggregate increase of 2.4%, or $420 million	CMS fact sheet
Medicare Advantage, 2027	Final rate increase of 2.48%, more than $13 billion	CMS final rate notice
Medicare telehealth	Specified Medicare flexibilities extended through December 31, 2027	HHS policy updates
Medicaid	Work requirements and six-month eligibility reviews for applicable expansion adults begin in 2027; provider-tax thresholds in most expansion states phase down from October 2027	Medicaid.gov; CMS provider-tax proposal
Marketplace coverage	Enhanced premium tax credits expired after 2025; enrollment fell from 22.1 million to 19.2 million	KFF, June 29, 2026
Pharmaceutical tariffs	100% default rate on covered patented products, effective July 31 for the companies named in Annex III and September 29 for others; country and company exceptions apply and generics are excluded	White House proclamation

State review of healthcare transactions now reaches private equity and management companies

California's AB 1415 took effect on January 1, 2026 and brings private equity groups, hedge funds and management services organizations into the state's transaction notice requirements (Nixon Peabody). Oregon's SB 951 bars management services organizations from majority ownership or control of medical practices, with existing arrangements required to comply by January 1, 2029 (Reed Smith). Massachusetts, Indiana, New Mexico, Rhode Island and Washington have enacted notice or approval requirements, and Maine's 180-day notice and approval regime begins January 1, 2027 (BakerHostetler).

Restrictive covenants for clinicians are also narrowing, and Virginia's ban on noncompetes for healthcare professionals, effective July 1, 2026, retains an exception for the sale of a business (DLA Piper). Federal tax law keeps a top long-term capital gains rate of 20% and an estate and gift exemption of $15 million per person (Tax Foundation; HBK CPAs & Consultants). A seller's proceeds also depend on entity and deal structure, the portion of the price taxed as ordinary income, the net investment income tax and state tax, and on the ownership and review requirements of each state involved.

POLICY AND PRICE

How reimbursement and trade policy reach the value of an individual business

Program-wide payment updates are the starting point for a buyer's analysis. Buyers then model the company's own payer contracts, billing codes, service mix and geography, because a Medicare Advantage plan payment update reaches providers only through their individual contracts and an aggregate home health estimate differs from the rate change at any one agency.

EXPOSURE	HOW BUYERS MODEL IT	WHAT AN OWNER CAN PREPARE
Physician services	Buyers model the proposed conversion-factor change together with code-level relative value units, site-of-service rules and commercial contracts, and they apply the higher factor only to clinicians who qualify under an advanced alternative payment model	A revenue bridge by code and payer that shows reimbursement, volume and wage sensitivities separately
Home health and outpatient	Buyers separate base-rate changes, adjustments, wage indices and procedure eligibility from the headline update	The historical service mix repriced under the proposed schedule, and updated when the final rule is issued
Medicaid and coverage	Eligibility changes affect applicable adults and state programs differently, and enrollment loss, authorization delays and uncompensated care can reduce revenue and collections	Exposure mapped by state and patient category, with the expansion-adult rules applied only to the patients and service lines they cover
Pharmaceutical manufacturing	Tariff treatment depends on product, origin and company agreement, so buyers ask for evidence of eligibility before they credit US capacity with a tariff advantage or a higher valuation	A schedule of exposure by product, customer and origin, the contractual pass-through terms and evidence of qualifying treatment

Pharmaceutical tariffs in detail

The April 2 proclamation sets a 100% default rate on covered patented products and ingredients, effective July 31 for the companies named in Annex III and September 29 for others. Approved onshoring treatment carries a 20% rate that rises to 100% on April 2, 2030. Specified products from the European Union, Japan, South Korea, Switzerland and Liechtenstein receive 15% unless a lower rate applies, and the United Kingdom framework starts at 10%, with any reduction contingent on a further agreement and notice. Companies with qualifying most-favored-nation pricing and onshoring arrangements pay zero through January 20, 2029, and further conditional product exclusions apply. Generics, including biosimilars, remain excluded (White House proclamation, clauses 3 to 6).

In an acquisition, the importer's eligibility and agreement terms, including any change-of-control provisions, determine whether a reduced rate carries over to the new owner, and buyers will ask to see the agreement itself.

Proposed rules remain proposed as of October 5, 2026.

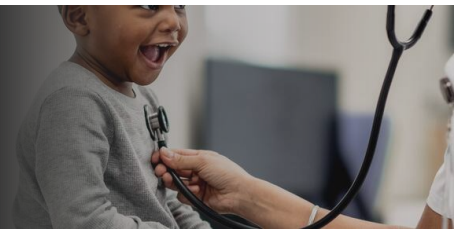
PROVIDER SERVICES

Health systems are buying access points and sponsors are trading platforms

14,655
Urgent care centers in the United States in April 2026

70
Urgent care centers HCA Healthcare acquired in three 2026 transactions

42.2%
Share of physicians working in physician-owned practices, down from 60.1% in 2012



Patient volumes are growing in the outpatient settings that founder-owned providers occupy, and HCA reported emergency room visits up 3.6% and equivalent admissions up 2.7% on a same-facility basis in the second quarter (HCA Healthcare). Concentra saw 2.6% more visits a day at 4.6% more revenue per visit (Concentra), and U.S. Physical Therapy averaged 33.5 visits per clinic per day at a net rate of $107.59, although therapist salaries rose to 57.9% of revenue from 56.6% (U.S. Physical Therapy).

Urgent care is consolidating toward hospital ownership, with the 100 largest operators holding 6,056 of the country's 14,655 centers and 55.9% of those locations affiliated with a hospital (Journal of Urgent Care Medicine, May 29, 2026). HCA bought 13 CommunityMed centers in North Texas in February, 17 Urgent Care Group clinics in the Carolinas in June and the 40-center Texas MedClinic in August (Becker's ASC Review; Healthcare Dive). Scale without hospital backing has been harder to sustain, and venture-backed Carbon Health filed for Chapter 11 in February with 96 clinics and more than $100 million of debt (Secured Finance Network).

In eye care, EyeCare Partners agreed on September 28 to sell its optometry business of more than 300 offices to MyEyeDr., with the proceeds going to debt reduction (EyeCare Partners). Independent practice continues to shrink, and 42.2% of physicians worked in physician-owned practices in the AMA's latest survey against 60.1% in 2012 (AMA survey via Medical Economics).

> *"The convergence of demographic demand, patient preference, technological innovation, and supportive policy is transforming home-based care from a niche offering into a core component of healthcare systems."*
>
> William Blair, The Home as the Future of Senior Care, September 29, 2026

Care in the home drew the largest provider transactions of the year, with Kinderhook agreeing to take Enhabit private at an enterprise value of about $1.1 billion (HLTH), and Addus HomeCare agreeing to pay about $275 million for AccentCare's personal care division, which generates about $280 million of annual revenue in ten states (Addus HomeCare). Addus grew personal care revenue 6.8% on a same-store basis with rate increases of 9.9% in Texas and 3.9% in Illinois (Addus second-quarter results).

Dialysis and emergency medical services are growing more slowly. DaVita reported normalized non-acquired treatment growth of 0.3% (DaVita), and Fresenius Medical Care reported US same-market treatments down 0.9% after exiting about 100 clinics (Fresenius Medical Care). Global Medical Response raised $478.7 million in its May initial public offering at a valuation of about $3.35 billion (JEMS), and Congress extended the Medicare ambulance add-on payments through December 31, 2027 (EMS1).

PROVIDER SERVICES: SELECTED ACQUISITIONS AND FINANCINGS

Disclosed prices are read alongside the ownership interest acquired

U.S. Physical Therapy paid $37.6 million for interests of 50% to 70% in three businesses with about $27.0 million of combined annual revenue. Those are prices for partial interests, so a comparison with a whole-company sale has to account for the share acquired, debt, the economics the founders retain and EBITDA (U.S. Physical Therapy; investor presentation, May 2026).

DATE	TARGET (LINKED)	BUYER	TERMS DISCLOSED
Sep 2026	EyeCare Partners optometry business	MyEyeDr.	More than 300 offices; price undisclosed; closing expected in the fourth quarter
Sep 2026	AccentCare personal care division	Addus HomeCare	About $275 million for about $280 million of annual revenue
Aug 2026	Texas MedClinic	HCA Healthcare	40 urgent care centers in San Antonio, Austin and Houston; undisclosed
Jul 2026	12-clinic physical therapy practice	U.S. Physical Therapy	$16.4 million for a 67% interest; $12.0 million of annual revenue
Jun 2026	Urgent Care Group	HCA Healthcare	17 clinics in the Carolinas; undisclosed
May 2026	Global Medical Response	Initial public offering	$478.7 million raised at a valuation of about $3.35 billion
Apr 2026	Haven Health Management	MKH Capital Partners	22 addiction and psychiatric locations; reported as a nine-figure price
Mar 2026	Talkspace	Universal Health Services	About $835 million
Feb 2026	Enhabit	Kinderhook	Enterprise value of about $1.1 billion; 249 home health and 117 hospice locations
Jan 2026	Eight-clinic physical therapy practice	U.S. Physical Therapy	$6.2 million for a 50% interest; $8.0 million of annual revenue
Jan 2026	PatientCare EMS Solutions	Grant Avenue Capital	Ground ambulance provider, Tyler, Texas; undisclosed
Jan 2026	Ally Pediatric Therapy	ACES, backed by General Atlantic	Pediatric and autism therapy, Phoenix; undisclosed

The reimbursement calendar bears on price in each of these trades, and Medicare's proposed 2027 dialysis rule would raise total payments 1.1% (CMS), and Acadia Healthcare, whose adjusted EBITDA fell to $149.2 million from $201.8 million, has named Medicaid work requirements and labor costs as risks to behavioral health volumes (Acadia Healthcare). Buyers are paying most readily for in-network commercial revenue, dense local footprints and clinicians who stay after closing.

PROVIDER SERVICES: WHAT THIS MEANS FOR BUYERS AND OWNERS

Each care model is valued on different evidence

CARE MODEL	WHAT SUPPORTS A PREMIUM	WHAT BUYERS TEST, AND WHO IS BUYING
Physician practice management and eye care	Clinician retention, organic visit growth, ancillary earnings, transferable contracts and a compliant ownership structure	Earnings and compensation by provider after closing, referral concentration and state ownership restrictions. Buyers include specialty platforms and MyEyeDr.
Surgery centers	Case mix, surgeon commitment, block utilization, payer rates and repeatable local density	Surgeon and referral concentration, facility investment and ownership economics. Buyers include health systems and specialist surgery center operators
Urgent and occupational care	Site-level contribution, density of access points, employer contracts and same-site demand	The ramp of new sites, staffing coverage, seasonality and episodic volumes. Buyers include HCA, Concentra and regional operators
Behavioral health and applied behavior analysis	Documented outcomes, authorized capacity, clinician retention and disciplined utilization	Medical necessity, coding, authorization denials and Medicaid exposure. Buyers include specialist strategic and sponsor-backed platforms
Physical therapy	Visits, net revenue per visit, therapist productivity and durable same-clinic growth	Wage pressure, patient acquisition and founder retention. Buyers include U.S. Physical Therapy and regional platforms
Home health	Referral diversity, case mix, collection quality and scalable clinical supervision	Exposure to Medicare's home health payment model, coding, eligibility and agency-level reimbursement. Buyers include home care platforms and sponsors
Hospice	Clinical quality, referral breadth and stable operating leadership	Eligibility documentation, length of stay, cap exposure and survey history. Buyers include specialist hospice and home care platforms
Personal care	Contract durability, caregiver capacity, density and alignment of wages with rates	State Medicaid rates, authorizations and labor availability. Buyers include Addus and regional care platforms

What this means for owners

A practice with $4 million of EBITDA, diversified referrals, durable clinician coverage and proven economics at new sites can attract interest as a platform. The same EBITDA produced largely by the founder, or resting on one fragile reimbursement stream, is more likely to be valued as an add-on or to carry contingent consideration. Owners who assemble provider, site, payer and referral schedules before setting price expectations enter those conversations better prepared, and a commercially insured patient mix supports value when realized rates, collections and retention support the earnings.

The buyers named are examples drawn from the results and transactions on the preceding pages.

LIFE SCIENCES & DIAGNOSTICS

Research spending has turned up and laboratories are selling to the national networks

1.22x
IQVIA net new business relative to revenue in the second quarter

$9.1B
Biotech venture funding in the first half, the most since early 2022

15%
Annual Medicare laboratory fee cut that could resume in 2027



Clinical laboratories are consolidating into Labcorp and Quest through hospital outreach purchases and sales by independent owners. Quest grew revenue 10.2% in the second quarter on 13.1% more requisitions (Quest Diagnostics). Labcorp spent $528.6 million on acquisitions in the first half, recording $250.0 million of acquired assets for Empire City Laboratories and $165.0 million for the Parkview Health outreach laboratory (Labcorp Form 10-Q), after paying about $194 million for Community Health Systems' outreach laboratories (Labcorp, December 2, 2025). Independent laboratories number 7,824 of the 301,284 CLIA-certified sites in the country (CMS, September 2026).

Medicare payment is the open question for laboratory owners, because fee cuts under the Protecting Access to Medicare Act have been delayed through 2026, and reductions of up to 15% a year could resume in 2027 unless Congress passes the RESULTS Act (Association for Diagnostics & Laboratory Medicine).

Outsourced research has recovered from two slow years, with IQVIA's net bookings up 19% (IQVIA), ICON reporting a 22% sequential rise in requests for proposal (ICON), Medpace growing revenue 17.2% (Medpace) and Charles River posting its best organic growth since 2023 (Charles River Laboratories). Biotech venture funding reached at least $9.1 billion in the first half (BioPharma Dive), and Thermo Fisher described the US academic and government market as stabilizing (Thermo Fisher call summary). Congress has kept the 15% cap on NIH indirect costs blocked, although the fiscal 2027 budget request proposes it again (Congressional Research Service).

> *"Biopharma spending and activity are picking up thanks to increasing specialty therapeutics demand, greater funding, and more drug approvals."*
>
> Harris Williams, Outsourced Pharma Services, September 24, 2026

Private equity sponsors continue to lead the consolidation of clinical research sites. Headlands Research, owned by THL, bought the two-site Clinical Trials Research in California in June (Headlands Research), and Summit Clinical Research added Dallas Research Institute to its Pinnacle network in September (Summit Clinical Research).

Trade policy changes manufacturing economics by product, origin and company agreement, and buyers verify tariff eligibility, pass-through and change-of-control terms (White House proclamation; detail on page 5). The BIOSECURE Act became law in December 2025 (Akin), and Medicare moved skin substitutes to a flat rate of about $127 per square centimeter in 2026, a change CMS expects to reduce spending by $19.6 billion (Polsinelli).

LIFE SCIENCES & DIAGNOSTICS: ANNOUNCED TRANSACTIONS

Strategic buyers paid for consumables, data and laboratory volume

The largest buyers of 2026 were operating companies adding recurring product lines. Danaher's Leica Biosystems bought StatLab, a pathology consumables maker that Audax and Linden had built through nine add-on acquisitions in five years, and Merck KGaA agreed to buy Bio-Techne at an enterprise value of $11.3 billion, a 36% premium.

DATE	TARGET (LINKED)	BUYER	TERMS DISCLOSED
Sep 2026	StatLab	Danaher (Leica Biosystems)	Anatomic pathology consumables; about $250 million of 2025 revenue; undisclosed
Sep 2026	Dallas Research Institute	Summit Clinical Research, backed by LongueVue Capital	Metabolic disease research site; undisclosed
Sep 2026	Worldwide Clinical Trials early-phase unit	Fortrea	$45 million for a 200-bed clinical pharmacology unit and laboratory in Texas
Jun 2026	Bio-Techne	Merck KGaA	Enterprise value of $11.3 billion; signed, closing expected by early 2027
Jun 2026	Clinical Trials Research	Headlands Research, owned by THL	Two physician-led sites in California; undisclosed
Jun 2026	Biocare Medical	Agilent	$950 million
Mar 2026	Clario	Thermo Fisher Scientific	$8.875 billion in cash plus deferred and contingent payments
Feb 2026	Empire City Laboratories	Labcorp	Founder-owned clinical laboratory, select assets; $250.0 million of assets recorded
Jan 2026	Catalyst Clinical Research	Worldwide Clinical Trials	Oncology-focused research organization; about $500 million
Jan 2026	Resolution Medical	Resonetics	Device design and manufacturing; undisclosed
Dec 2025	Community Health Systems outreach laboratories	Labcorp	About $194 million in cash for assets in 13 states

Contract development and manufacturing saw its own large trades, including Samsung Biologics' $1.8 billion agreement for peptide manufacturer PolyPeptide and Lone Star Funds' purchase of Lonza's capsules business, alongside new US capacity from Cambrex, PCI and Vetter (DCAT Value Chain Insights). Device design firms sold to manufacturers that want engineering in house, as the Resonetics and Currier purchases show, and NAMSA bought Labcorp's US medical device testing business in January (Medical Product Outsourcing).

LIFE SCIENCES & DIAGNOSTICS: WHAT THIS MEANS FOR BUYERS AND OWNERS

Testing volume, research backlog and recurring products are valued separately

BUSINESS MODEL	WHAT SUPPORTS A PREMIUM	WHAT BUYERS TEST, AND WHO IS BUYING
Clinical and outreach laboratories	Durable ordering relationships, test mix, accreditation and profitable collection of billed revenue	Reimbursement by code, medical necessity, denials and customer concentration. Buyers include Labcorp and Quest
Clinical research sites	Repeat sponsor awards, investigator depth, enrollment performance and geographic density	Study concentration, investigator departures, cancellations and pass-through revenue. Buyers include networks such as Headlands and Summit
Contract research and research services	Quality of executable backlog, repeat clients and sustainable staff utilization	Conversion of bookings to cash, cancellation rights, biotech funding and project profitability. Buyers include research strategics and specialist sponsors
Contract development and manufacturing	Validated processes, repeat contracted work, capacity utilization and required quality standards	Customer and program concentration, transfer costs, capital spending, inspection history and tariff eligibility by product. Buyers include manufacturing strategics and sponsors
Tools, reagents and pathology consumables	Repeat purchasing, an installed base, differentiated chemistry and a place in the customer's workflow	Durability of intellectual property, customer qualification, substitution and margin after manufacturing investment. Buyers include Danaher, Thermo Fisher and Merck KGaA
Device development and testing	Regulatory expertise, repeat programs and engineering depth beyond the founder	Project dependence, utilization and transferability of expertise. Buyers include strategic networks such as Resonetics and NAMSA

What this means for owners

A recurring consumables company earns its premium by demonstrating repeat orders, customer qualification and durable gross margin. A research business demonstrates executable backlog and continuity of investigators or technical staff, and a laboratory bridges billed revenue to cash by code and payer. Presenting the business on the evidence that fits its model serves an owner better than a single life sciences median, which conceals the reason a buyer would pay a premium.

Exposures to resolve before going to market

Laboratory owners can model the scheduled 2027 reductions to the Clinical Laboratory Fee Schedule test by test, which gives a more accurate result than a 15% cut applied to total revenue (CMS proposed rule). Research businesses can distinguish funded awards from proposals and cancellable work. Manufacturers can quantify the investment needed to fulfill committed production, because buyers value contracted earnings and give little credit for unused capacity.

The buyers named are examples drawn from the results and transactions on the preceding pages.

MEDICAL PRODUCTS & DISTRIBUTION

Procedure demand is firm and consumables are outgrowing capital equipment

9.0%
Stryker organic sales growth in the second quarter

$36.5B
Medtech deal value in the first half, after a decade-high 2025

41%
Health system executives who plan to cut capital spending in 2026



Device makers are reporting steady procedure volumes, with Stryker growing 9.0% organically in the second quarter (Stryker), Medtronic raising its organic growth guidance to between 7.25% and 7.75% (Medtronic) and Zimmer Biomet growing 4.0% (Zimmer Biomet). Hospitals are spending more readily on consumables than on installed equipment, and Steris grew healthcare service revenue 10% and consumables 9% while capital equipment grew 1% (Steris), and 41% of health system executives plan to cut capital spending (HFMA).

The same preference for consumables shows in instrument reprocessing, where Aspen Surgical bought Ruhof Healthcare, a maker of instrument-cleaning detergents with about $50 million of revenue, in January (Aspen Surgical).

"A wide variety of healthcare products and services create strong investor appeal due to their nondeferrable and recurring demand."

Harris Williams, Outlook 2026: Healthcare & Life Sciences

Dental results diverged between distributors and manufacturers, as Henry Schein grew dental merchandise distribution 9.7% (Henry Schein) and Envista grew 5% (Envista via Pulse 2.0), while Dentsply Sirona's sales fell 4.1% with the Americas down 10.7% (Dentsply Sirona). Mitsui Chemicals agreed to buy Ultradent Products for $900 million (Becker's Dental Review).

Several of the largest distribution businesses changed ownership in the past year. Medline raised $6.26 billion in its December initial public offering and grew net sales 11.6% in the second quarter (Fierce Healthcare; Medline). Owens & Minor received $375 million in cash, a retained equity stake and a preferred return for its products and healthcare services segment (Owens & Minor), McKesson sold 13% of its medical-surgical unit for $1.25 billion (McKesson) and Cardinal Health spent $360 million on two home supply businesses (HME News).

In respiratory and home equipment, Teleflex sold its acute care and urology businesses to Intersurgical for $530 million and its component unit to Montagu and Kohlberg for $1.5 billion (MedTech Dive), Viemed grew revenue 23.9% on home ventilation (HME News) and Medicare is restarting competitive bidding for home medical equipment with contracts from 2027 (AAHomecare).

Tariffs have moved earnings up as well as down. The Supreme Court struck down the emergency-powers tariffs in February and importers began receiving refunds in April (Barnes Dennig), after which Medline booked a net benefit of $243 million and Stryker reversed $158 million of charges. The Section 232 investigation into medical devices remained open in September with no tariff imposed (iData Research), and the FDA's Quality Management System Regulation took effect on February 2 (Ropes & Gray).

MEDICAL PRODUCTS & DISTRIBUTION: SELECTED ACQUISITIONS AND FINANCINGS

Sponsors bought manufacturing platforms and strategics bought product lines

KKR's agreement to take contract manufacturer Integer Holdings private values the company at $5.7 billion, a 51.8% premium, and Danaher paid about 18 times estimated 2027 EBITDA for Masimo. Divestitures supplied much of the remaining activity, and Bain & Company counts them as 34% of strategic medtech deal value in 2025 (Bain & Company via Fierce Biotech).

DATE	TARGET (LINKED)	BUYER	TERMS DISCLOSED
Sep 2026	Numotion	Hanger, owned by Patient Square Capital	Complex rehabilitation equipment; undisclosed; closing expected in the fourth quarter
Aug 2026	Integer Holdings	KKR	Enterprise value of $5.7 billion; $127 a share
Jul 2026	AdaptHealth diabetes business and Strive Medical	Cardinal Health	$360 million combined
Jun 2026	Ultradent Products	Mitsui Chemicals	Dental consumables; $900 million
Jun 2026	Masimo	Danaher	Enterprise value of about $9.9 billion; 18x estimated 2027 EBITDA
Jun 2026	McKesson Medical-Surgical, 13% interest	Apollo	$1.25 billion
May 2026	Amplitude Vascular Systems	Stryker	$435 million plus up to $400 million in milestones
Mar 2026	Apex Dental Laboratory Group	Private equity investors	16 dental laboratories in 12 states; undisclosed
Jan 2026	Ruhof Healthcare	Aspen Surgical	Instrument reprocessing chemistries; about $50 million of revenue; undisclosed
Dec 2025	Medline	Initial public offering	$6.26 billion raised at a valuation above $38 billion
Dec 2025	Owens & Minor Products & Healthcare Services	Platinum Equity	$375 million in cash, a 5% retained equity interest and a preferred return
Dec 2025	Teleflex acute care and urology; component unit	Intersurgical; Montagu and Kohlberg	$530 million; $1.5 billion

Owens & Minor's announced terms combined $375 million of cash with a 5% retained equity interest and a preferred return, and the company also kept specified tax assets, so the cash figure alone understates what the seller received (Owens & Minor). Comparing distribution with manufacturing valuations requires the full consideration, the scope acquired and comparable earnings. Buyers of product companies will examine clearances, quality records, customer qualification and the exposure of the bill of materials to tariffs.

MEDICAL PRODUCTS & DISTRIBUTION: WHAT THIS MEANS FOR BUYERS AND OWNERS

Protected economics matter more than the size of the business

BUSINESS MODEL	WHAT SUPPORTS A PREMIUM	WHAT BUYERS TEST, AND WHO IS BUYING
Proprietary devices	Protected technology, clinical adoption, required clearances and a durable installed base	Product concentration, reimbursement, competing technology and post-market quality. Buyers include strategics such as Stryker and Danaher
Contract manufacturing	Qualified programs, recurring production and capabilities that are difficult to replace	Customer concentration, requalification risk, scrap, capacity and maintenance capital spending. Buyers include manufacturing strategics and specialist sponsors
Consumables and reprocessing	Repeat purchase demand, clinical preference and recurring use in the customer's workflow	Substitution, pricing pressure, supplier risk and support for product claims. Buyers include Aspen Surgical and product strategics
Dental products and laboratories	Repeat demand, proprietary products or meaningful local laboratory density	Exposure to discretionary procedures, dentist concentration and technician retention. Buyers include dental strategics and consolidators
Medical-surgical distribution	Exclusive access, customer retention, route density and reliable conversion of working capital to cash	Purchasing-group pricing, inventory obsolescence, vendor concentration and commodity margins. Buyers include distributors and operationally focused sponsors
Home equipment and supplies	Recurring service or supply demand, collection discipline and geographic density	Payer authorization, equipment utilization, competitive bidding and compliance. Buyers include Hanger, Cardinal Health and specialist platforms

What this means for owners

A manufacturer substantiates its regulatory status, quality history and the durability of customer qualification. A distributor shows contractual access, retention and cash conversion after inventory investment. Both benefit from identifying tariff exposure and presenting sustainable earnings after one-time refunds or reversals, because buyers will discount high recurring revenue when margins, working capital or customer access are fragile.

Reading the Owens & Minor transaction

The $375 million cash payment was accompanied by a 5% retained interest and preferred equity economics, and the seller also kept specified tax assets (Owens & Minor announcement). A comparison across sub-sectors therefore requires the full consideration, comparable EBITDA, debt, the scope acquired and normalized cash conversion.

The buyers named are examples drawn from the results and transactions on the preceding pages.

PHARMACY & HEALTHCARE SERVICES

Drug pricing reform is reshaping pharmacy while staffing returns to growth

$606B
US net medicine spending in 2025, up 10.6%

$39.0B
Forecast US healthcare staffing revenue in 2026

$250M
Sale price of Omnicare, which CVS bought for about $13 billion in 2015



Medicine spending is growing quickly while pharmacy margins shrink. Net spending reached $606 billion in 2025, up 10.6%, with GLP-1 and obesity drugs adding $14 billion (IQVIA Institute). Independent pharmacies numbered 18,960 in mid-2025 and reported their lowest gross profit in ten years (NCPA Digest), and 67% of owners surveyed report waiting 22 days or more for manufacturer refunds on drugs with negotiated Medicare prices (Drug Topics).

Negotiated Medicare prices have cut deepest in long-term care pharmacy, where Guardian Pharmacy Services grew revenue 2% while serving 8% more residents, and its chief executive said first-half revenue would have grown at a low double-digit rate without the price reductions (Guardian Pharmacy Services; call summary). BrightSpring attributed about $50 million of second-quarter pressure in its home and community pharmacy unit to the same cause (BrightSpring call summary), and a bankruptcy court approved the sale of Omnicare to GenieRx for $250 million in cash (CVS Health; Becker's Hospital Review).

Specialty pharmacy and infusion are growing fastest. BrightSpring's specialty and infusion revenue rose 30% to $2.9 billion (BrightSpring), and Option Care Health reported high-single-digit growth in acute therapies and named tuck-in acquisitions as a priority (Option Care Health; call summary). Neither of the bills to create a full Medicare home infusion benefit has been enacted (RxToolKit).

Congress enacted pharmacy benefit manager reform in February, under which Part D compensation must be delinked from list prices from 2028 and rebates passed through in full to employer plans from 2029 (Sidley Austin), and all three large managers have settled the FTC's insulin case on terms that include cost-plus pharmacy reimbursement (Truveris). State laws barring managers from owning pharmacies remain in litigation (Mintz).

Drug distributors continue to buy physician practice management organizations. Cencora completed its purchase of OneOncology in February (Cencora), and The Specialty Alliance, which Cardinal Health controls, acquired Solaris Health for about $1.9 billion in cash in November 2025, leaving Cardinal with about 76% of the alliance after closing (Cardinal Health Form 10-K). McKesson grew oncology and multispecialty revenue 33% (McKesson).

Healthcare staffing is growing again after three years of contraction, and Staffing Industry Analysts forecasts 1% growth to $39.0 billion in 2026 and 3% in 2027, led by locum tenens at 4% (Staffing Industry Analysts). AMN Healthcare grew nurse and allied revenue 11% (AMN Healthcare), and health care and social assistance had 1,359,000 open positions in August (Bureau of Labor Statistics JOLTS).

PHARMACY & HEALTHCARE SERVICES: ANNOUNCED TRANSACTIONS

Scale mergers met resistance and buyers turned to specialist tuck-ins

Aya Healthcare ended its agreement to buy Cross Country Healthcare in December 2025 after an extended FTC review (Staffing Industry Analysts), and Knox Lane completed a $437 million purchase of the company in July at $13.25 a share against the $18.61 Aya had offered (Fierce Healthcare). Purchases of smaller specialist firms in pharmacy, infusion, perfusion and locum tenens closed throughout the year.

DATE	TARGET (LINKED)	BUYER	TERMS DISCLOSED
Sep 2026	Nautilus Pharmacy	Guardian Pharmacy Services	Tennessee pharmacy assets serving 14 states; undisclosed
Aug 2026	Life Couriers	JLL Partners	Radiopharmaceutical and time-critical healthcare logistics; undisclosed
Jul 2026	Cross Country Healthcare	Knox Lane	$437 million; nurse, allied and locum tenens staffing
Jul 2026	Wellness Concepts	Guardian Pharmacy Services	Long-term care pharmacy, Virginia; undisclosed
Jun 2026	Quest Locum Tenens and Syncx	VeloSource, backed by Interlock Equity	Locum tenens staffing and workforce software; undisclosed
Jun 2026	BluHaven Management and Realo Specialty Care Pharmacy	Soleo Health	Infusion and specialty pharmacy; undisclosed
May 2026	Omnicare	GenieRx Holdings	$250 million in cash through a bankruptcy sale; long-term care pharmacy in 47 states
Apr 2026	Ohio Valley Perfusion Associates	Strata Critical Medical	About $1 million; a mid-single-digit EBITDA multiple
Feb 2026	OneOncology	Cencora	Oncology practice management; completed
Feb 2026	IDR Healthcare	Care Career	Travel nurse, allied and school staffing; the buyer's sixth purchase in 18 months
Jan 2026	Focus Staff	Elite365	Travel nurse and allied staffing, Dallas; undisclosed

Buyers in this focus area say they pay for contracted, recurring revenue and for spare capacity they can fill. Strata Critical Medical cited recurring revenue, multi-year contracts and high customer retention in perfusion services, and Guardian describes pharmacy acquisitions as a way to use the capacity of its existing pharmacies. Staffing Industry Analysts counted 23 healthcare staffing transactions in 2025 against 17 in 2019, with buyers favoring diversified workforce platforms (Staffing Industry Analysts). Travel nurse gross margin of 19.9% in 2025 remains below the roughly 25% earned before the pandemic (Staffing Industry Analysts).

PHARMACY & HEALTHCARE SERVICES: WHAT THIS MEANS FOR BUYERS AND OWNERS

Buyers underwrite gross profit, cash conversion and contract durability

BUSINESS MODEL	WHAT SUPPORTS A PREMIUM	WHAT BUYERS TEST, AND WHO IS BUYING
Specialty pharmacy and infusion	Durable payer and manufacturer access, clinical capability and patient retention	Net margin by therapy, access to limited-distribution drugs, authorizations, inventory and receivables. Buyers include Option Care Health and Soleo Health
Long-term care pharmacy	Facility retention, service quality, operating capacity and profitable resident growth	Effects of negotiated prices, facility concentration, rebates and reimbursement lag. Buyers include Guardian and regional operators
Locum tenens	Credentialing quality, repeat facility demand and durable clinician relationships	Specialty concentration, fill rates, compliance and dependence on individual recruiters. Buyers include specialist platforms such as VeloSource
Travel and per diem staffing	Recurring accounts, workforce diversity and margin at normal bill rates	Crisis revenue, spread compression, payroll funding and client churn. Buyers include platforms such as Care Career and Elite365
Perfusion and specialist outsourced care	Multi-year contracts, retention and dependable clinical coverage	Hospital concentration, change-of-control consents and specialist retention. Buyers include Strata Critical Medical
Healthcare logistics	Reliable service levels, network density and specialized handling	Customer concentration, quality failures and required network investment. Buyers include strategic and sponsor-backed platforms such as Life Couriers

What this means for owners

Pharmacy revenue growth can reflect drug prices or an expensive therapy mix without producing equivalent gross profit or cash, and staffing revenue can recover while wage spreads remain weak. Owners in these businesses are best served by leading with a bridge from revenue to gross profit, EBITDA and cash, by showing contract retention, reimbursement lag and working capital needs, and by separating recurring specialist demand from strike, crisis and pandemic activity.

Physician alignment is part of the price

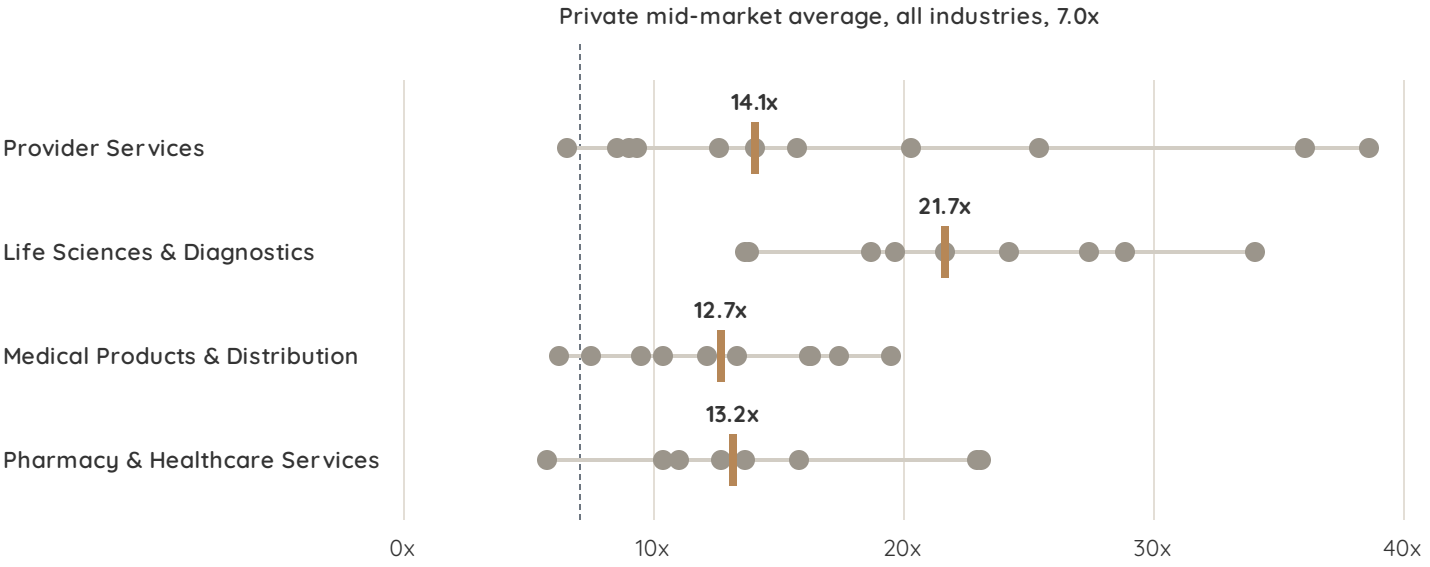
The Specialty Alliance acquired Solaris Health in November 2025 for approximately $1.9 billion in cash, and Cardinal Health owned approximately 76% of the alliance after closing. The alliance also issued units to physicians and management, and Cardinal's filing accounts for part of that value as compensation after the combination, so the units cannot simply be added to the cash price to estimate enterprise value (Cardinal Health Form 10-K, Note 2).

The buyers named are examples drawn from the results and transactions on the preceding pages.

PUBLIC-MARKET VALUATION REFERENCE

Public multiples show relative standing, and private prices are set business by business

Enterprise value to trailing twelve-month EBITDA, listed companies by focus area; each dot is one company and the gold bar marks the median



These are enterprise values of public companies relative to reported trailing EBITDA. Differences in sub-sector, size, growth, earnings adjustments and business mix mean the chart cannot be discounted directly to a founder-owned company. The 13.6x median that private equity paid for healthcare services companies in 2025 (PitchBook via Becker's ASC Review), the 7.0x all-industry average for the second quarter of 2026 (GF Data via ACG) and the 5.8x median for transactions of $5 million to $50 million (IBBA and M&A Source Market Pulse) each describe a different sample and period, so they serve as separate reference points. Page 19 sets out how public trading, strategic control transactions, sponsor platforms and private add-ons relate to the value of a private company.

Provider Services: HCA Healthcare 9.1x; Tenet Healthcare 6.6x; U.S. Physical Therapy 15.8x; DaVita 8.5x; Addus HomeCare 12.7x; Pennant Group 25.4x; Acadia Healthcare 9.4x; LifeStance Health 36.1x; Privia Health 38.7x; Concentra 14.1x; Ensign Group 20.4x

Life Sciences & Diagnostics: Labcorp 13.7x; Quest Diagnostics 13.8x; Thermo Fisher 24.3x; Danaher 21.7x; Bio-Techne 34.0x; IQVIA 18.7x; Medpace 27.5x; ICON 28.9x; Charles River 19.7x

Medical Products & Distribution: Stryker 16.3x; Steris 13.3x; Teleflex 19.5x; Henry Schein 12.1x; Dentsply Sirona 7.5x; Envista 10.4x; Zimmer Biomet 9.5x; Integer Holdings 16.2x; Solventum 17.4x; Viemed Healthcare 6.2x

Pharmacy & Healthcare Services: Option Care Health 11.0x; BrightSpring 23.0x; Guardian Pharmacy 23.1x; McKesson 15.8x; Cencora 12.7x; Cardinal Health 13.7x; CVS Health 10.4x; AMN Healthcare 5.7x

Multiples are enterprise value to trailing EBITDA as published by StockAnalysis and retrieved on October 5, 2026; they use reported EBITDA, which is lower than the adjusted EBITDA companies present, and an unusually low EBITDA can inflate a ratio. Select Medical, Cross Country Healthcare and NeoGenomics are excluded because the first two were acquired during 2026 and the third has minimal trailing EBITDA.

VALUATION: FROM REFERENCE DATA TO A PRIVATE COMPANY

A private company's value is built from matched evidence

EVIDENCE	WHAT IT SHOWS	WHAT MAKES IT COMPARABLE
Public trading	Relative market sentiment toward minority, liquid shares, expressed as enterprise value	A matching sub-sector and EBITDA definition, with allowance for control premiums, size and growth
Strategic precedents	What specific acquirers paid for control or for assets	Separation of synergies, partial interests, earnouts, distressed sales and retained economics
Sponsor platforms	Pricing for businesses that can support a standalone investment	Matching scale, management, leverage, growth and recurring cash generation
Private add-ons	Value to an existing operating platform	Integration can remove overhead and scarcity can raise the price, so an add-on can sell at or above platform pricing
Founder-owned private transactions	The closest pricing evidence when business model, size and risk align	Verified consideration and earnings the buyer accepted, with all-industry surveys as context

EBITDA SCALE	THE QUESTION BUYERS ASK	EVIDENCE THAT ANSWERS IT
$2 million to $5 million	Whether the business can stand alone as an investment or fits best inside a larger platform	The cost of replacing the owner, clinician and customer concentration, and finance and compliance infrastructure
$5 million to $10 million	Whether the operating model can sustain growth without the founder	Management depth, unit economics by site or customer, repeatable growth and capital needs
$10 million and above	Whether scale translates into a durable platform	Leadership, reporting, integration capability and cash conversion across locations

Illustration: the earnings a buyer accepts drive proceeds

Assume a company reports adjusted EBITDA of $7.0 million, and a buyer deducts $0.6 million for the recurring cost of replacing the owner and $0.4 million of earnings it considers unsustainable, leaving $6.0 million of accepted EBITDA. At an assumed multiple of 8.0x, enterprise value is $48.0 million. After $8.0 million of net debt and $2.0 million of seller fees, proceeds before tax are $38.0 million in a full cash sale with no other adjustments. Valuing the unadjusted $7.0 million at the same multiple would overstate enterprise value by $8.0 million.

All figures in the illustration are hypothetical, and 8.0x is an arithmetic assumption. The size bands describe how buyers frame their questions and do not represent pricing tiers. Earnouts, rollover equity, escrows, leases and working capital adjustments also affect the cash received at closing.

Benchmark International's activity in these focus areas

Benchmark International has closed 12 transactions in these four focus areas over the trailing 24 months and is working on 39 further engagements as of October 1, 2026. Pharmacy and healthcare services is the largest group among the closed transactions, with five, followed by life sciences and diagnostics with four. Provider services leads the live engagements with 19, of which ten are in preparation for market. Across all four, 20 engagements are on the market, 13 are in preparation, four are in legal documentation and two are in discussion with a buyer.

Among the nine announced transactions listed below, six went to operating companies, one to a private equity firm and one to a group of private investors, and in one the buyer is undisclosed.

SELLER (LINKED)	BUYER AS ANNOUNCED	WHAT THE SELLER DOES	FOCUS AREA
Precise Bioscience	New Horizon Medical Solutions	Amniotic membrane allografts for wound care and sports medicine, Durham, North Carolina	Life Sciences & Diagnostics
Formulation Technology	AAVIN Private Equity	Contract manufacturer of nutritional supplements, Oakdale, California	Life Sciences & Diagnostics
Fast Response On-Site Testing	Relentless Health	Mobile occupational health testing, Santa Cruz, California	Life Sciences & Diagnostics
Critical Environments Professionals, certification division	Scientific Safety Alliance	Certification testing for pharmacy compounding spaces, laboratories and bio-containment facilities	Life Sciences & Diagnostics
Dental Holdings	Solmetex	Patent holder and brand owner of ReLeaf hands-free dental suction devices	Medical Products & Distribution
FosterBridge	Amivie	In-home care for Ohio Medicaid waiver participants	Provider Services
Medical Temps	Health Advocates Network	Nurse staffing for healthcare facilities in northern Louisiana	Pharmacy & Healthcare Services
The Brooks Group and Associates	A group of private investors	Training, research and instructional design for pharmaceutical, biotech and device companies, West Chester, Pennsylvania	Pharmacy & Healthcare Services
Fitzgerald Enterprises	An undisclosed buyer	Healthcare executive and clinical placement	Pharmacy & Healthcare Services

What buyers reward and what they discount

Across the four focus areas, acquirers pay the most for revenue that recurs under contract or clinical necessity and that is paid by a balanced mix of payers, and they discount revenue that depends on one reimbursement rate, one referral source or the owner personally. The specifics differ by focus area, and an owner or advisor can test a business against them well before a sale.

FOCUS AREA	BUYERS REWARD	BUYERS DISCOUNT
Provider Services	In-network commercial revenue and a balanced payer mix; dense local footprints; clinicians who stay, often with retained equity; clean billing and coding records; capacity to open new sites	Heavy dependence on one Medicaid program or one fee schedule; referral concentration; restrictive covenants that state law no longer enforces; structures that need restructuring under new state ownership rules
Life Sciences & Diagnostics	Recurring consumables and reagents; accreditation and certification scope; hospital and physician relationships that route test volume; sponsor and research organization repeat business; US manufacturing capacity	Revenue tied to one Medicare payment code; dependence on one grant-funded customer group; a single sponsor or study; capacity that needs reinvestment
Medical Products & Distribution	Patents and regulatory clearances; consumables sold on a recurring basis; clinical relationships and brand preference; quality system records ready for inspection	Tariff-exposed supply chains without alternatives; commodity distribution without exclusive lines; pricing set by one purchasing group; capital equipment sold into shrinking hospital budgets
Pharmacy & Healthcare Services	Specialty and infusion therapies with limited distribution access; multi-year facility contracts; high customer retention; credentialed clinicians in specialist roles such as perfusion and locum tenens	Brand drug volume subject to negotiated prices; legal exposure in billing practices; commodity per diem staffing; revenue that depends on strike or crisis demand

In the second quarter of 2026, 87% of surveyed transactions above $5 million received at least three offers, and lower-middle-market sales took about 11 to 12 months to close (IBBA and M&A Source Market Pulse). That survey covers all industries, and a healthcare sale can also require state notices or approvals, payer consents, enrollment actions and a billing and coding review, depending on structure and jurisdiction. Owners who prepare financial, concentration, license, accreditation and management evidence before approaching buyers shorten that path.

SELLER READINESS: TIMING AND PROCEEDS

Should an owner transact now, prepare or wait?

The decision rests on expected net proceeds and execution risk for the owner's own business, and the quarter's largest transaction or a public-company median says little about either. An owner's desired liquidity, future role, control, clinician continuity and patient-care objectives come first, and they shape the choice of buyer and process.

PATH	WHEN IT IS COMPELLING	WHAT MUST BE TRUE
Launch a process	Durable earnings, retention and reporting can already be demonstrated, and several buyers have a credible strategic reason to engage	The earnings hold up through a quality-of-earnings review, and contracts, licenses and regulatory timelines support a closing
Prepare for 6 to 12 months	A discrete, achievable improvement in reporting, retention, a contract renewal or a seasoned group of sites can reduce a visible discount	The improvement is measurable, and the expected increase in proceeds exceeds the cost of preparation and the risk of delay
Wait and retain ownership	Important growth or reimbursement outcomes remain unresolved, or current terms fall short of the owner's economic and operating goals	Remaining independent is adequately funded and attractive after capital spending, working capital and exposure to downside scenarios
Consider a partial sale	The owner wants liquidity and continued participation, or a partner can provide capital and operating infrastructure	Rollover valuation, governance, dilution, distributions, exit rights and compensation are acceptable on their own terms, apart from the headline price

What buyers will request first

Financial: three years of monthly statements, trailing results, an EBITDA bridge supported by evidence, revenue, margin and cash by site or service, and schedules of debt, leases, capital spending and working capital.
Commercial and clinical: payer, customer and referral concentration, contracts and their change-of-control terms, clinician employment, compensation, retention and credentialing, and growth and retention by patient or customer cohort.
Regulatory: licenses, accreditation, billing and coding reviews, audit history, quality metrics, privacy and cybersecurity controls, and the transaction requirements of each state involved.

Measuring the cost of waiting

A sale today can be compared with a later sale weighted by its probability, including interim cash distributions, the investment in preparation, taxes and the risk of lower earnings or pricing. A projected increase in EBITDA adds value for the owner only when it exceeds the capital the owner must fund and the added uncertainty of a later exit, and the comparison is most useful when it includes both a base case and a downside case for reimbursement and retention.

Statutory review periods, payer consents and enrollment requirements depend on jurisdiction, transaction structure and the specific contracts.

BENCHMARK INTERNATIONAL MIDDLE MARKET

Considering a transaction in healthcare?

Our first step is always a confidential conversation exploring your goals, your company and your options.



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Sources

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MEDICAL PRODUCTS & DISTRIBUTION

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PHARMACY & HEALTHCARE SERVICES

IQVIA Institute; NCPA Digest; Drug Topics; Guardian Pharmacy Services; call summary; BrightSpring call summary; CVS Health; Becker's Hospital Review; BrightSpring; Option Care Health; call summary; RxToolKit; Sidley Austin; Truveris; Mintz; Cencora; Cardinal Health Form 10-K; McKesson; Staffing Industry Analysts; AMN Healthcare; Bureau of Labor Statistics JOLTS; Staffing Industry Analysts; Fierce Healthcare; Nautilus Pharmacy; Life Couriers; Cross Country Healthcare; Quest Locum Tenens and Syncx; BluHaven Management and Realo Specialty Care Pharmacy; Ohio Valley Perfusion Associates; IDR Healthcare; Focus Staff; Staffing Industry Analysts

VALUATION AND SELLER PREPARATION

PitchBook via Becker's ASC Review; GF Data via ACG; IBBA and M&A Source Market Pulse; IBBA and M&A Source Market Pulse

BENCHMARK INTERNATIONAL

Transaction records and the engagement portfolio as of October 1, 2026; announced transactions as published at benchmarkintl.com/insights and linked in the table on page 20; listed-company multiples from StockAnalysis, each linked on page 18

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